



Infection Prevention and Control Manual

Clostridioides difficile Policy

Scope

Site	Service/Department/Unit	Disciplines
SCGHOPHCG	Organisation Wide	All Staff

Introduction

Clostridioides difficile (*C. difficile*) is a toxin producing gram-positive spore-forming anaerobic bacterium which can cause illness ranging from mild diarrhoea to potentially life-threatening pseudomembranous colitis.¹ Hypervirulent/epidemic strains of *C. difficile* infection (CDI) have been reported worldwide and have been associated with outbreaks in healthcare facilities with significant mortality due to the severity.^{2,3} CDI is the most commonly recognised cause of healthcare related diarrhoea.²

Risk Statement

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGHOPHCG Mission & Values	☒	Staff Safety	☒
Other:	☐		

Policy Principles

The purpose of this policy is to describe the infection prevention and management (IPM) procedures at Sir Charles Gairdner and Osborne Park Hospitals to minimise the transmission of *C. difficile*.

Definitions

<i>C. difficile</i> colonisation	The patient tests positive for <i>C. difficile</i> organism and/or its toxin but exhibits <u>NO</u> clinical symptoms (patient is asymptomatic).
<i>C. difficile</i> infection	<i>C. difficile</i> infection (CDI) is defined by the presence of symptoms (usually diarrhoea), and either a faecal specimen positive for <i>C. difficile</i> toxins or toxigenic <i>C. difficile</i> , or colonoscopic or histopathological findings revealing pseudomembranous colitis. ²

1. Risk factors for acquisition

The main risk factors for acquisition are:

- Advanced age
- Residence in aged care facilities
- Patients who have prolonged hospitalisation
- Patients who are immunocompromised
- Patients with recurrent or current antibiotic exposure
- Patients with gastric acid suppressive therapy or surgery

2. Mode of transmission for *C. difficile*

C. difficile is transmitted via the faecal-oral route by:

- **Direct contact:** contaminated hands of healthcare personnel^{2, 3}
- **Indirect contact:** contact with contaminated medical and patient care equipment (e.g., commodes) or environmental surfaces. Spores, the infective form, can persist on environmental surfaces for months.²

3. Detection and diagnosis

C. difficile infection (CDI) is defined by the presence of symptoms (usually diarrhoea), and either a faecal specimen positive for *C. difficile* toxins or toxigenic *C. difficile*, or colonoscopic or histopathological findings revealing pseudomembranous colitis.²

CDI should be suspected at the onset of diarrhoea if it cannot be attributed to another cause, such as medicines other than antibiotics, especially if the patient has one or more risk factors for acquisition ([refer to Section 1](#)). Diarrhoea is classified as stool loose enough to “take the shape of a specimen container” or as Bristol Stool Scale (Bowel Assessment Chart), types 5 – 7.

A stool specimen must be collected and sent to the laboratory immediately. The negative predictive value of a negative toxin PCR test is high (96-99%), however a second test may be warranted if there is a strong clinical suspicion of CDI.²

Routine screening of patients and testing of stool specimens from asymptomatic patients is not recommended. Patients may remain colonised for several months, therefore, **DO NOT** repeat testing for *C. difficile* if the patients’ symptoms have resolved.

NOTE: The IPM Unit should be contacted if two or more patients or staff report diarrhoea within a 24-hour period. Other causes such as norovirus should also be considered. Testing for norovirus by PCR must be specifically requested.

4. Management of patients with *C. difficile* infection

Early diagnosis and prompt isolation with the use of contact precautions is essential to minimise the transmission of *C. difficile*. This includes any patient with suspected CDI and symptomatic patients who test positive for the *C. difficile* organism but are toxin negative.^{2,3}

Contact precautions includes:

- Single room with ensuite facilities.^{2, 3}
- An orange “CONTACT PRECAUTIONS” sign to be placed on door.
- Limit patient room changes within the ward or transfer between wards.³
- Use of personal protective equipment, including gloves and gown, prior to entry of room^{2, 3}
- Refer to Standard and Transmission Based Precautions Policy.

NOTE: standard precautions apply at all times

4.1 Hand Hygiene

- Perform hand hygiene in accordance with the ‘5 moments for hand hygiene’.
- Use handwash solution or alcohol-based hand rub. If hands are visibly soiled, washing with soap and water is required.^{2, 3}
- Encourage patient personal hygiene i.e., handwashing and showering to reduce the burden of spores on the skin.²

NOTE: In an outbreak setting washing hands with soap and water is recommended when caring for patients with CDI.^{2, 4}

4.2 Environmental Cleaning

Preventing contamination of the environment is an important strategy in preventing transmission of *C. difficile*. This includes:

- Daily environmental cleaning: Follow the routine cleaning procedures for patients on contact precautions.
- Discharge cleaning: Following patient discharge, a disinfection clean to the single room is required. At SCGH this will be followed by additional disinfection with hydrogen peroxide (Nocospray®). Equipment such as commodes should remain in the room during the additional disinfection with hydrogen peroxide (Nocospray®). Privacy curtains and toilet brushes are to be disposed.⁶

Refer to the Infection Prevention and Control Environmental Cleaning Policy.

5. Duration of Contact Precautions

Contact precautions should remain in place until the patient has had no diarrhoea for at least 48 hours. Patients should be monitored daily for frequency and severity of diarrhoea using the Bristol Stool Scale. Diarrhoea is defined as types 5 – 7. A bowel assessment chart must be used to document bowel activity. Re-testing for *C. difficile* within the same episode of CDI is not required to determine clearance before removing the patient from isolation.³ If symptoms recur after the initial episode has fully settled (usually 2 weeks) then a repeat specimen is indicated to confirm recurrent CDI.

Compliance, monitoring and evaluation

The SCGOPHCG IPM Unit meets the requirements of the Mandatory Policy MP OD0108/19 Healthcare Associated Infection Surveillance in Western Australia (HISWA) Policy. All cases of hospital identified CDI are reported to the HISWA.

Related Documents

- National Safety and Quality Health Service Standard Preventing and Controlling Infections
- Mandatory Policy Healthcare Associated Infection Surveillance in Western Australia Policy
- SCGOPHCG IP&M Manual:
 - Standard and Transmission Based Precautions Policy
 - Hand Hygiene Policy
 - Environmental Cleaning Policy
 - Reprocessing of Reusable Medical Devices
 - Manual: Cleaning of Patient Care Equipment
- SCGOPHCG Cleaning Guidelines

References

1. Strategies to Prevent Clostridium difficile Infections in Acute Care Hospitals: 2014 Update by the Society for Healthcare Epidemiology of America (SHEA) and the Infectious Diseases Society of America (IDSA). Infection Control & Hospital Epidemiology [Internet]. 2014, [cited 2024 Feb 28];35(6): 628-645. Available from <https://www.cambridge.org/core/services/aop-cambridge-core/content/view/D5ADD87BD88203703AF85707CE799F6E/S0899823X00193857a.pdf/strategies-to-prevent-clostridium-difficile-infections-in-acute-care-hospitals-2014-update.pdf>
2. Johnson S, Lavergne V, Skinner W, Gonzales-Luna A, Garey K, Kelly C, Wilcox M. Clinical Practice Guidelines for the Management of Clostridioides difficile Infection in Adults:2021 Update by SHEA/IDSA [Internet]. 2021;[cited 20/03/24]. Available from <https://www.idsociety.org/practice-guideline/clostridioides-difficile-2021-focused-update/>

3. Australian Commission of Safety and Quality in Health Care. Clostridium difficile infection. A model to improve the management and control of Clostridium difficile in Australia. March 2018, [cited 2024 Feb 28]. Available from <https://www.safetyandquality.gov.au/publications-and-resources/resource-library/clostridium-difficile-infection-model-improve-management-and-control-clostridium-difficile-australia>
4. Australian Commission of Safety and Quality in Health Care. Implementation Guide for the Surveillance of Clostridioides difficile infection - [cited 2024 Feb 28]. Available from https://www.nhmrc.gov.au/about-us/publications/australian-guidelines-prevention-and-control-infection-healthcare-2019-09/surveillance_guide_cdi_august_2023.pdf
5. National Health and Medical Research Council. Australian guidelines for the prevention and control of infection in healthcare. [Internet]. Canberra (Australia): National Health and Medical Research Council; 2019. [cited 2024 Feb 28]. Available from <https://www.nhmrc.gov.au/about-us/publications/australian-guidelines-prevention-and-control-infection-healthcare-2019>
6. Government of Western Australia North Metropolitan Health Service. Sir Charles Gairdner Osborne Park Health Care Group. Environmental Cleaning Standards: April 2023, [cited 2024 March 27]. Available from <https://scgophcghealthpoint.hdwa.health.wa.gov.au/directory/Corporate/Nursing/PatientSupportServices/Cleaning%20Services/Documents/Cleaning%20Guidelines%20April%202023.pdf>

Authorisation & Version Control

Policy Sponsor	Executive Sponsor Infection Prevention and Management Committee				
Policy Contact	Coordinator of Nursing, Infection Prevention and Management				
Date First Issued:	01/08/1986	Last Reviewed:	20/08/2024	Review Date:	20/08/2027
Version No.	5				
Endorsed by:	SCGOPHCG Infection Prevention and Management Committee			Date:	11/11/2024
Standards Applicable:	 				

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form.

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